

STARLIGHT INSURANCE

HEALTH PROTECT FAMILY FLOATER POLICY - POLICY SCHEDULE

Document Type: Copy of Policy Schedule & Benefits Summary **Status:** SAMPLE/TRAINING COPY ONLY

Policy Number	HEA567890	Plan Name	Health Protect Family Floater - Gold
Policyholder	Vikram Desai	Relationship to Patient	Son
Insured Member / Patient	Ramesh Chandra Desai	Age / Gender	68 years / Male
Policy Period	01-04-2025 to 31-03-2026	Issue Date	29-03-2025
Sum Insured	Rs 10,00,000	Co-payment	10% for insured members aged 60+
Network Eligibility	Cashless treatment available at network hospitals, subject to authorization	TPA / Assistance Desk	Starlight Health Assist Desk

Insured Members

Name	Relationship	Date of Birth / Age	Coverage Status
Vikram Desai	Primary Policyholder	12-08-1988 / 36 years	Active
Ramesh Chandra Desai	Father / Dependent Parent	14-02-1957 / 68 years	Active

Key Coverage Benefits

Benefit	Coverage / Limit	Remarks
In-patient hospitalization	Covered up to Sum Insured	Minimum 24 hours hospitalization, except listed day-care procedures
Emergency cardiac care	Covered under in-patient emergency hospitalization	Includes ICU, emergency investigations, procedures and approved consumables as per policy terms
ICU / Critical care charges	Covered up to eligible room/ICU limits	Subject to medical necessity and hospital billing review
Coronary angioplasty / stent placement	Covered when medically necessary	Stent charges payable as per approved package/rate and applicable regulations
Pre-hospitalization expenses	30 days	Relevant diagnostic and consultation expenses linked to admission
Post-hospitalization expenses	60 days	Follow-up medicines/investigations related to admitted condition

Important Conditions

This policy summary indicates active coverage for the named insured member during the policy period. Cashless authorization is subject to verification of active policy status, identity, admissibility, exclusions, waiting periods, policy limits, co-payment, and receipt of hospital documents. Emergency hospitalization must be notified to the insurer/TPA as soon as reasonably possible.

Emergency claim note: Acute myocardial infarction and emergency angioplasty are generally treated as emergency hospitalization events when supported by clinical records. Final admissibility is subject to claim assessment.

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POLICY TERMS SNAPSHOT - SAMPLE COPY

Exclusions / Waiting Period Snapshot

Item	Policy Position
Pre-existing disease waiting period	Applicable as per underwriting schedule unless waived/modified by endorsement. Emergency stabilization is reviewed as per policy wording.
Specific disease waiting period	Applicable only where listed in the full policy wording. Acute emergency cardiac events are assessed based on documents and policy terms.
Non-medical items	Payable/non-payable as per standard admissibility rules and hospital bill review.
Experimental or unproven treatment	Excluded unless specifically approved under policy terms.
Fraud / misrepresentation	Claims may be rejected if documents or declarations are found false, altered, or misleading.

Claim / Cashless Authorization Requirements

- Duly completed pre-authorization request from network hospital.
- Copy of policy schedule or e-card and government photo ID of insured member.
- Admission notes, emergency/cardiology notes, ECG, troponin reports and procedure notes.
- Angiogram/angioplasty report, stent stickers, implant invoice and itemized hospital bill.
- Discharge summary and final bill at the time of discharge or settlement.

Contact and Authorization Desk

Customer Care	1800-000-0000 (sample)
Email	claims@starlight.example (sample)
Cashless Desk Hours	24 x 7 for medical emergencies
Hospital Network Note	Apollo Hospital Delhi listed as sample network hospital for this training scenario

Authorized Signatory

For Starlight Insurance - Policy Services Department

DISCLAIMER: This is a generated sample document for claim workflow testing/training. It is not issued by Starlight Insurance or any real insurer, does not create coverage, and must not be submitted as an authentic insurance document.